A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 April 2026

Dr Chandrani Jayasekara
Parkwood Green Medical

Dear Dr Jayasekara,

Re: Miss Thisumi Walliwala
DOB: 2005-09-19
Address: 20 Torino Avenue, GREENVALE VIC 3059

Summary

- **Gastroscopy and colonoscopy revealing no histological evidence of inflammatory bowel disease, Coeliac disease, or Helicobacter pylori.**
- **For specialised intestinal ultrasound at St Vincent's Hospital to evaluate the small bowel in the context of elevated faecal calprotectin.**
- **Management of tenesmus and dysmotility through dietary modification, including kiwi fruit and high-fibre intake.**
- **Recommendation for pelvic floor physiotherapy.**
- **Action required: Please consider implementing a Chronic Disease Management Plan to facilitate Thisumi's access to pelvic floor physiotherapy.**

I had the pleasure of conducting a telehealth review with Thisumi Walliwala to discuss the results of her recent gastroscopy and colonoscopy. You will be pleased to know that the investigations were largely reassuring.

While a small ulcer was noted macroscopically during the colonoscopy, biopsies of this lesion and the surrounding colonic mucosa were histologically normal, with no evidence of active inflammation or microscopic colitis. Furthermore, the gastroscopy excluded Coeliac disease and *Helicobacter pylori* infection. From a technical perspective, there was some difficulty intubating the terminal ileum, which can occasionally suggest a history of prior inflammation; however, there is currently no definitive evidence of Crohn's disease. While the likelihood of inflammatory bowel disease has decreased significantly, it cannot be entirely excluded at this stage.

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Given Thisumi's elevated faecal calprotectin, it is necessary to evaluate the four metres of small intestine that remain inaccessible via standard endoscopy. I have therefore referred her for a specialised intestinal ultrasound at St Vincent's Hospital to ensure no proximal pathology has been overlooked.

Thisumi continues to experience bothersome gastrointestinal symptoms, including alternating bowel habits, urgency, and significant tenesmus. We discussed the high prevalence of pelvic floor dysfunction in patients with a history of endometriosis and how this may be contributing to her sensation of incomplete evacuation. To address this, I have advised her to optimise her defaecation mechanics by using a footstool to improve the anorectal angle and to focus on a high-fibre morning routine involving porridge and a hot drink to stimulate the gastrocolic reflex. I have also recommended the daily consumption of two kiwi fruits to assist with bowel consistency.

Impression and Plan My clinical impression is that Thisumi's symptoms likely relate to a combination of dysmotility and pelvic floor dysfunction, though we must complete small bowel imaging to rule out Crohn's disease. I have encouraged her to seek a review with a pelvic floor physiotherapist to assist with retraining the pelvic floor musculature. Regarding her symptoms of reflux and regurgitation, I have provided a prescription for Nizatidine to be used on an as-needed basis. I have opted to defer any neuromodulators for the time being, focusing instead on these conservative mechanical and dietary interventions. I will review Thisumi again once the intestinal ultrasound results are available.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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